INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Dacarbazine — metastatic melanoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00464
Version 3

Reference treatment cycle

Treatment stages

01 **Treatment visits**
21-day reference cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

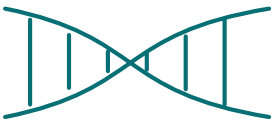
What each medicine does

To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

CYTOTOXIC CHEMOTHERAPY

Dacarbazine

DTIC



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Dacarbazine is converted in the body into substances that damage DNA and make it harder for cancer cells to divide. It also affects some healthy cells, including blood-forming cells. This explains why nausea prevention and blood-count checks are central to the plan.

Damages cancer-cell DNA → Treatment plan and safety checks → Report new symptoms early

What to watch for

Report fever or feeling acutely unwell, bleeding, persistent vomiting, severe abdominal pain or burning at the infusion site.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Record where the melanoma began, its pathology result and the sites involved now. This pathway treats melanoma that cannot be fully removed or has spread. Treatment after complete removal has a different aim and a separate plan.

The tumour test

The laboratory checks a sample of the melanoma for a BRAF V600 change. The result can identify a target for certain medicines. A change found in the tumour does not, by itself, establish an inherited condition.



Scans and local treatments

Add planned scans, any operation and any radiotherapy to the same calendar. Ask what each part is intended to achieve and which team arranges it.

New changes still matter

Tell your team about a changing mole, a persistent sore, a new lump or an unexplained symptom. Improvement in a known tumour does not explain a new problem elsewhere.

Your medicines and overall plan

The schedule

Dacarbazine by intravenous (IV) infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Important boundaries

This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

Evidence and choices

The dacarbazine comparator response rate was 13.9%; this is not a validated efficacy estimate for NCCP dacarbazine 850 mg/m². These are group results from that study, not a prediction for you. Dacarbazine dose differs from NCCP 00464, and the population was BRAF wild type. The study primarily established benefit of nivolumab over a historical comparator. No exact survival or response prediction is assigned to the NCCP 850 mg/m² course.

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Support alongside treatment

Supportive care addresses symptoms and daily life during cancer treatment. Palliative care can be part of that support; accepting it does not mean you must stop anticancer treatment.

A person you trust

Choose whether to involve a relative or supporter and what may be shared with them. Ask for an interpreter or a different information format. Check that you can explain the next dose, tests and urgent-contact plan in your own words.

Your document set

Keep the current treatment summary, medicine list, test plan and contact card together. Use the printed instructions when you need them; the QR code is an optional link to further information.

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment phases

Dacarbazine — metastatic melanoma

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY REFERENCE CYCLE

Treatment visits

Treatment visits **Repeat only as prescribed**

Dacarbazine by intravenous (IV) infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00464 v3: current regimen source · HPRA Dacarbazine 200 mg product information, March 2024

THE WHOLE TREATMENT JOURNEY

Dacarbazine — metastatic melanoma: treatment and checks

Advanced melanoma

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00464	Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans.	Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.	Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.	Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00464	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Written plan and contacts NCCP 00464	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1 **Treatment visits**

21-day reference cycle

To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The NCCP dose is dacarbazine 850 mg/m² on day 1 in one litre of saline over one hour, with the infusion protected from light. The individual dose is prescribed using body size and clinical review. Allow extra time for blood results, anti-sickness medicine, preparation and any other fluids. Tell the nurse immediately about pain, burning, swelling or redness around the infusion.

STEP 2

Missed, vomited or held treatment

If treatment is delayed for low blood counts, illness or another problem, the team sets the new date and dose. Do not mark a planned but cancelled dose as received. The six-cycle limit is conditional on benefit and tolerability, not a requirement to give unsafe treatment.

STEP 3

Appointments and supply

Before each visit, check four things: the blood-test booking, how you will get there, your medicine supply and any help you need. Ask who will review the results and tell you whether the next dose can go ahead.

STEP 4

Paper copies and contacts

Keep the handbook, short guide and treatment card in one place. Ask your team to fill in the real daytime and emergency numbers and print a replacement when the plan changes. A phone is optional.

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Practical arrangements

Write down where blood samples are taken, who receives the results and how you obtain the next appointment or medicine supply. Request a printed plan if that is easier to use.

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.

A treatment summary

Ask for a treatment summary showing what you received, important side effects, admissions, the last dose and the follow-up team. Share the current version with your GP and other clinicians.

Your next review

Before leaving, confirm the next test and review, how you will receive results and whom to call sooner. Keep those numbers available after treatment ends.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment visit	 **Day 1** The NCCP dose is dacarbazine 850 mg/m² on day 1 in one litre of saline over one hour, with the infusion protected from light. The individual dose is prescribed using body size and clinical review. Allow extra time for blood results, anti-sickness medicine, preparation and any other fluids. Tell the nurse immediately about pain, burning, swelling or redness around the infusion.
Between-visit monitoring	 **Days 2–21** Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.

Schedule and review: Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Holds, missed treatment and phase changes: If treatment is delayed for low blood counts, illness or another problem, the team sets the new date and dose. Do not mark a planned but cancelled dose as received. The six-cycle limit is conditional on benefit and tolerability, not a requirement to give unsafe treatment.

> **Why these bands stay broad:** This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00464 v3: current regimen source ↗ · HPRA Dacarbazine 200 mg product information, March 2024 ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1: Treatment day

The NCCP dose is dacarbazine 850 mg/m² on day 1 in one litre of saline over one hour, with the infusion protected from light. The individual dose is prescribed using body size and clinical review. Allow extra time for blood results, anti-sickness medicine, preparation and any other fluids. Tell the nurse immediately about pain, burning, swelling or redness around the infusion. Report symptoms and previous reactions before treatment.

Day 2–21: Between treatment visits

Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling. Do not wait for the next appointment to report urgent symptoms.

Support around the medicines

1 Supportive medicines and symptom plan

Dacarbazine has high nausea-and-vomiting risk, so a planned preventive anti-sickness regimen is needed under the local protocol, including clear take-home instructions. The NCCP line that routine premedication is usually unnecessary does not remove this antiemetic requirement. Ask what to take regularly, what is rescue treatment and when to call if it fails.

2 Skin and daily care

Hair loss and sun sensitivity can occur, though they are not inevitable. Use gentle skin care and sun protection. Report redness, pain or swelling at a previous infusion site, or a spreading rash or signs of an allergic reaction.

3 Fertility, pregnancy and breastfeeding

The current HPRA product information advises effective contraception during treatment and for six months after the final dose for women, and during treatment and for three months afterwards for men. Discuss fertility preservation and, when relevant, genetic counselling before treatment. Do not breastfeed during treatment; ask the team about the appropriate interval afterwards.

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans.

During treatment

Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

What to expect between visits

A quiet treatment day does not rule out later problems.



Supportive medicines

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

When life is affected

Bring up difficulties with eating, sleep, pain, energy, work or caring for someone. The team can arrange symptom support while cancer treatment continues.

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Food, fluids and activity

Choose food, fluids and movement that you can manage, within any personal restrictions. Ask for help early if you are losing weight, drinking less or finding everyday activity harder.

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sexual health

You can request a private discussion about intimacy, sexual symptoms or relationships. Ask for help if the agreed pregnancy precautions are difficult to follow.

Questions worth asking

Which tests are needed?

Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans. Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed. Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

The treatment this guide covers This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Dacarbazine — metastatic melanoma · support plan

Active medicines: Dacarbazine

Emetogenic risk: High per NCCP 00464 v3; preventive antiemetic regimen required under local policy

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 Current NCIS V8 non-AC high-risk row; review interactions.
Ondansetron	16 mg orally	Day 1 Review QT risk, electrolytes, constipation and hepatic dosing.
Dexamethasone	12 mg orally	Day 1 Reconcile the complete steroid plan and aprepitant interaction.
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, glucose and interactions.
Aprepitant	80 mg orally once daily	Days 2–3 Check the take-home schedule.
Dexamethasone	8 mg orally once daily	Days 2–4 Non-AC high-risk schedule; do not substitute the AC row.
Metoclopramide	10 mg orally up to three times daily as needed	Breakthrough / subsequent days; duration to be prescribed Caution with olanzapine because of additive extrapyramidal effects; review renal function, contraindications and alternative rescue options.

Additional phase-specific support

- Use an individual approved high-risk antiemetic prescription, with prevention and breakthrough instructions. The source-based medicines below are a reviewable reference, not an automatic prescription; reconcile interactions, steroid exposure, sedation, QT risk and rescue alternatives.
- Dacarbazine has high nausea-and-vomiting risk, so a planned preventive anti-sickness regimen is needed under the local protocol, including clear take-home instructions. The NCCP line that routine premedication is usually unnecessary does not remove this antiemetic requirement. Ask what to take regularly, what is rescue treatment and when to call if it fails.

- Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.
- Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.
- The current HPRA product information advises effective contraception during treatment and for six months after the final dose for women, and during treatment and for three months afterwards for men. Discuss fertility preservation and, when relevant, genetic counselling before treatment. Do not breastfeed during treatment; ask the team about the appropriate interval afterwards.
- This is single-agent dacarbazine at the NCCP 850 mg/m² dose. It is not an immune-checkpoint or BRAF-targeted regimen. Historical studies often used 1000 mg/m² and different treatment settings; their response or survival figures are not a precise forecast for this 850 mg/m² course.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy / concentration

Dacarbazine

Tiredness, anaemia, poor sleep and nausea can affect concentration. Report severe dizziness, confusion or a major change in function; do not drive if unwell.

3 Breathlessness / anaemia

Dacarbazine

Low red cells can cause tiredness or breathlessness. New or worsening breathlessness needs assessment; severe breathlessness or chest pain is an emergency.

5 Liver

Dacarbazine

Rare severe liver injury can cause abdominal pain, fever and jaundice. Seek urgent assessment; routine liver tests do not replace symptom reporting.

7 Stomach / appetite

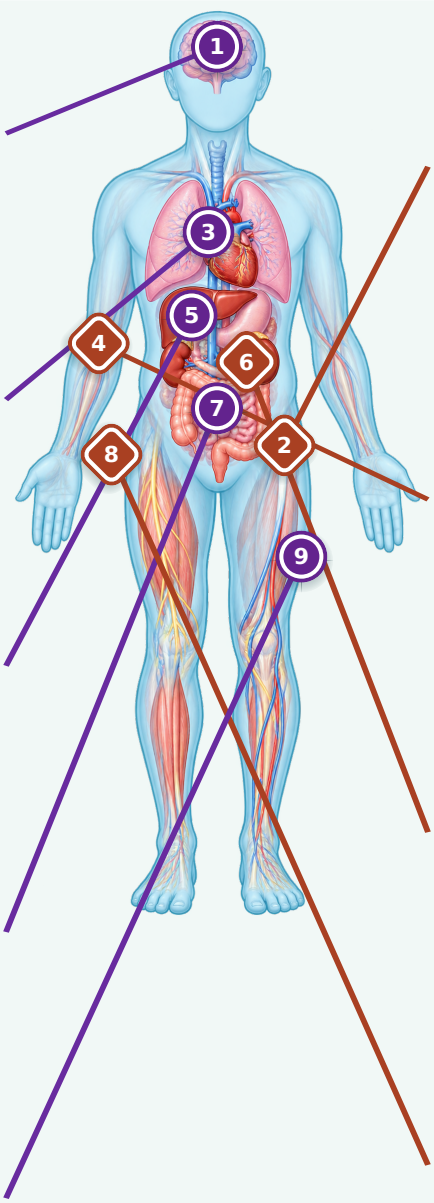
Dacarbazine

Nausea and vomiting need preventive medicines and a take-home plan. Report persistent symptoms, severe abdominal pain or failure of rescue treatment.

9 Bruising / bleeding

Dacarbazine

Low platelets can cause bruising, nosebleeds or bleeding gums. Persistent bleeding, black stools or blood in urine require prompt assessment.



2 Blood / infection

Dacarbazine

Low white cells increase infection risk and may occur late in the cycle. Fever or suddenly feeling unwell needs urgent contact, even near the next planned dose.

4 Skin / hair

Dacarbazine

Hair loss or sun sensitivity can occur. New severe rash, swelling or breathing difficulty may indicate allergy and needs urgent help.

6 Kidneys / fluids

Dacarbazine

Vomiting and reduced intake can lead to dehydration. Report reduced urine, persistent vomiting or inability to drink; kidney tests also help guide dosing.

8 Infusion site

Dacarbazine

Pain, burning, swelling or redness during infusion can mean leakage outside the vein. Tell the nurse immediately because tissue injury is possible.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Follow your unit’s fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00464 v3: current regimen source ↗**
   https://assets.hse.ie/media/documents/464_V3_Dacarbazine.pdf
2. **HPRA Dacarbazine 200 mg product information, March 2024 ↗**
   https://assets.hpra.ie/products/Human/19327/Licence_PA0623-003-002_04032024123758.pdf
3. **Robert et al. CheckMate 066, NEJM 2015 — historical context only**
   https://pubmed.ncbi.nlm.nih.gov/25399552/
4. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
5. **NCI: melanoma — patient information**
   https://www.cancer.gov/types/skin/patient/melanoma-treatment-pdq
6. **NCI: melanoma treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/melanoma-treatment-pdq
7. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
8. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
9. **NCCP NCIS medical-oncology antiemetics V8, Table 1 p4, high risk excluding AC**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf